

ANNUAL PMTCT DATABASE MONITORING REPORT

**Reporting Period:
January – December 2025**

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Indicators

Indicator	Definition
1	No. of pregnant women who delivered.
1a	No. of pregnant women who delivered with at least one ANC visit for current pregnancy.
2	No. of live births.
3	No. of pregnant women who had an abortion, stillbirth (SB) or neonatal death (PND = up to 6 weeks post-delivery).
4	No. of pregnant women who had a 1st ANC visit for the current pregnancy.
5	No. of pregnant women who did not have documented ANC visits and delivered a live birth (no known ANC visits, no 3rd trimester test result, HIV/STI risk, inadequate documentation of Syphilis cure).
5a	Of the number in 5 how many were tested for HIV and Syphilis.
5b	Of the number in 5a, how many tested positive.
6	No. of pregnant women who delivered a SB and were tested for Syphilis and confirmed positive.
7	Total no. of pregnant women tested for Syphilis (among the women that delivered).
7a	No. of pregnant women tested for Syphilis (among the women that delivered).
8	Of the number in question 7, how many pregnant women were confirmed for Syphilis (SD Bioline screened positive and TRUST Titre).
9	Of the number in question 8, how many pregnant women were appropriately treated for PMTCT (One dose Benzathine Penicillin (2.4mU IM) at least 30 days prior to delivery).
10	No. of infants with confirmed Congenital Syphilis.
11	Total No. of pregnant women who delivered and tested for HIV (including: known positive status, tested during pregnancy, and/or during childbirth/postpartum ≤ 72 hours after birth).
12	No. of pregnant women who delivered and tested for HIV (including: known positive status, tested during pregnancy, and/or during childbirth/postpartum ≤ 72 hours after birth).
12A	Of the number in question 12, how many pregnant women who delivered tested POSITIVE for HIV (including: known positive status, tested during pregnancy, and/or during childbirth/postpartum ≤ 72 hours after birth).
13	Of the number in question 12, how many pregnant women who delivered tested POSITIVE for HIV (including: known positive status, tested during pregnancy, and/or during childbirth/postpartum ≤ 72 hours after birth).
13A	Of the number in question 13, how many pregnant women were KNOWN POSITIVES.
14	No. of HIV positive pregnant women who delivered (TOTAL IN 14A MUST BE EQUAL TO 13A).
14A	Mode of delivery for HIV positive pregnant women who delivered (TOTAL IN 14B MUST BE EQUAL TO 13A).
15	No. of HIV positive pregnant women who delivered and who received ARVs to reduce the risk of MTCT (ALL KNOWN AND NEW POSITIVES).
15A	Of the number in 15a, how many positive pregnant women delivered and received HAART to reduce the risk of MTCT.
15B	No. of HIV positive pregnant women who delivered and did not get ARVs.
16	Total number of SCIs on register.
17	Total of SCIs retained in care (new and previously confirmed).

Indicator	Definition
17A	Total of SCIs in Care including new and those retained in care who were adequately (receiving ten days of IV Cryspen) treated.
18	No. of SCIs receiving repeat titres to monitor for downward trend.
18A	No. of SCIs who needed repeat titres at greater than 1-year-old.
19	No. of confirmed cases of SCIs discharged from care (YTD).
20	No. of deaths among SCIs.
21	Total New HEIs (≤ 18 months).
22	No. of new HEIs whose mothers received ART during pregnancy.
23	No. of HEIs presenting post-delivery (Mother diagnosed HIV positive post-delivery).
24	No. of new HEIs who received ARVs at birth (≤ 72 hours) to reduce MTCT.
25	No. of new HEIs who received ARVs in the first 6 weeks of life.
26	No. of HEIs born alive and did not get ARVs.
27	HEIs started on Bactrim prophylaxis at ≤ 8 weeks or older.
28	No. of HEIs receiving the 1st PCR at birth to determine HIV status for the year.
29	No. of 18-month old HEIs completing 3, 5, 12, and 18 months visits.
30	No. of HEIs newly discharged from HIV care.
31	No. of HEIs lost to follow-up prior to completing assessments of their HIV status.
32	No. of deaths among HEIs excluding HEI that had already been diagnosed as HIV positive prior to death).
33	No. of HEIs seen.
34	No. of HEIs whose feeding practices were assessed and recorded.
34A	No. of HEIs who were breast fed exclusively.
34B	No. of HEIs who were formula fed exclusively.
34C	No. of HEIs who were mixed fed.
35	No. of HEIs who had a PCR/VL test to determine their HIV status.
35A	No. of HEIs who tested positive from the 1st PCR test to determine their HIV status.
35B	No. of HEIs who tested negative from the 1st PCR test to determine their HIV status.
36	No. of HEIs who had a HIV antibody/serological test to determine their HIV status.
36A	No. of HEIs who tested positive from a HIV antibody/serological test to determine their HIV status.
36B	No. of HEIs who tested negative from a HIV antibody/serological test to determine their HIV status.
37	Total number of children linked to Programme (HEIs and HIV-positive children) including those who are lost to follow-up.
38	Total number of children in Care (HEIs and HIV-positive children) including new and those retained in care.
39	Total number of new children in Programme (HEIs and HIV-positive children).
40	Current Grand Total of HIV-positive children in care (including those newly diagnosed above).
41	Current total no. of HIV-positive children/ adolescents on HAART.
41A	Total no. of children/adolescents newly initiated on HAART.
42	No. of HIV-positive children transferred to another site/Migrated.
43	No. of HIV-positive children transferred in from another site.
44	No. of deaths among HIV-positive children/adolescents.
44A	No. of deaths among HIV-positive children/adolescents.

Executive Summary

This report reviews the Prevention of Mother-to-Child Transmission (PMTCT) database for the reporting year 2025, focusing on antenatal care (ANC) coverage, HIV and syphilis testing and treatment, HIV-exposed infants (HEIs), HIV-positive children and adolescents, and the completeness and quality of reporting across all Regional Health Authorities (RHAs). Nationally, ANC coverage was high, with nearly all women who delivered reporting at least one ANC visit, reflecting strong utilization of maternal health services. However, several parishes recorded inconsistencies between delivery and ANC indicators, including missing or incorrectly reported data and failure to disaggregate ANC visits by gestational age, most frequently in South East Regional Health Authority (SERHA) and Western Regional Health Authority (WRHA).

- **HIV testing coverage** at delivery fell slightly below the 95% target, with **94%** of women knowing their HIV status. Most HIV-positive pregnant women received antiretroviral therapy to reduce the risk of mother-to-child transmission, indicating effective linkage between diagnosis and treatment. Despite this, inconsistencies between related HIV indicators were observed in multiple parishes, alongside periods of non-reporting that resulted in missing data across several indicators.
- **Syphilis testing coverage** was also generally high (**95%**), though monthly fluctuations were evident. Data quality issues persisted, particularly for Indicator 7a, which requires trimester-specific reporting. Among women diagnosed with syphilis, treatment coverage did not reach optimal levels, revealing missed opportunities for timely treatment and congenital syphilis prevention.

A review of the HEI indicators identified inconsistencies between reported HIV-positive deliveries and subsequent infant services. In some instances, HEI services were reported despite no HEIs being recorded, complicating interpretation of the early infant diagnosis cascade. Reporting on HIV-positive children and adolescents also revealed gaps, including missing indicators, incomplete monthly submissions, and periods of non-reporting. Overall, persistent data quality issues continue to affect effective programme monitoring through the database. In response to these discrepancies, updates were made to the PMTCT database, and PMTCT nurses were given a defined period to review the updates and provide feedback using a demonstration environment. The finalized updates will be rolled out and become effective in January 2026.

Database Updates

- EMTCT indicators for 2026 reporting were updated, aligning the database with revised national and global reporting requirements such as Global AIDS Monitoring (GAM) and Spectrum.
- New validation rules, to improve data quality, consistency and completeness was added.

Recommendations

- Strengthen routine data quality checks and verification processes before report submission across all parishes.
- Improve syphilis and PMTCT indicator reporting by enhancing coordination with laboratories and standardizing trimester-specific data capture.
- Align HIV-positive delivery records with ARV administration data through regular cross-checks and enforced internal reporting deadlines.
- Address late, missing, and incomplete reports by introducing system reminders, clear accountability, and performance recognition for timely submissions.
- Reinforce review and feedback mechanisms at parish and national levels to identify and resolve data gaps prior to reporting cycles.
- Conduct targeted case reviews to improve syphilis treatment classification and coverage.

Introduction

A review of the PMTCT database revealed that for the period January to December a total of 123 of 156 (Figure 1) reports were submitted, resulting in an overall completeness rate of 79%. The Western Regional Health Authority (St. James, Trelawny, Hanover, and Westmoreland) is expected to submit 12 reports per quarter, amounting to a total of 48 reports for the year. The North East Regional Health Authority (St. Ann, St. Mary, and Portland), the Southern Regional Health Authority (St. Elizabeth, Manchester, and Clarendon), and the South East Regional Health Authority (Kingston and St. Andrew, St. Thomas and St. Catherine) are each expected to submit nine reports per quarter, amounting to 36 reports annually, respectively (Figure 1). A review of the database revealed that the North East Regional Health Authority had the highest overall submission rate at 97%, followed by the Western Regional Health Authority at 88%, the Southern Regional Health Authority at 81%, and the South East Regional Health Authority at 47%. Overall compliance with report submission for 2025 was 78%. Data completeness during the latter part of the year may have been adversely affected by the impact of a catastrophic Category Five hurricane that struck the island in October 2025. The hurricane caused significant damage to health facilities, widespread power outages, and interruptions in internet connectivity, which limited real-time data entry and access to the PMTCT database particularly in heavily affected parishes within WRHA and SRHA. Additionally, displacement of health centre staff and patients further disrupted routine updates and health-seeking behaviours, potentially impacting early detection and reporting of pregnant mothers for the October to December 2025 period.

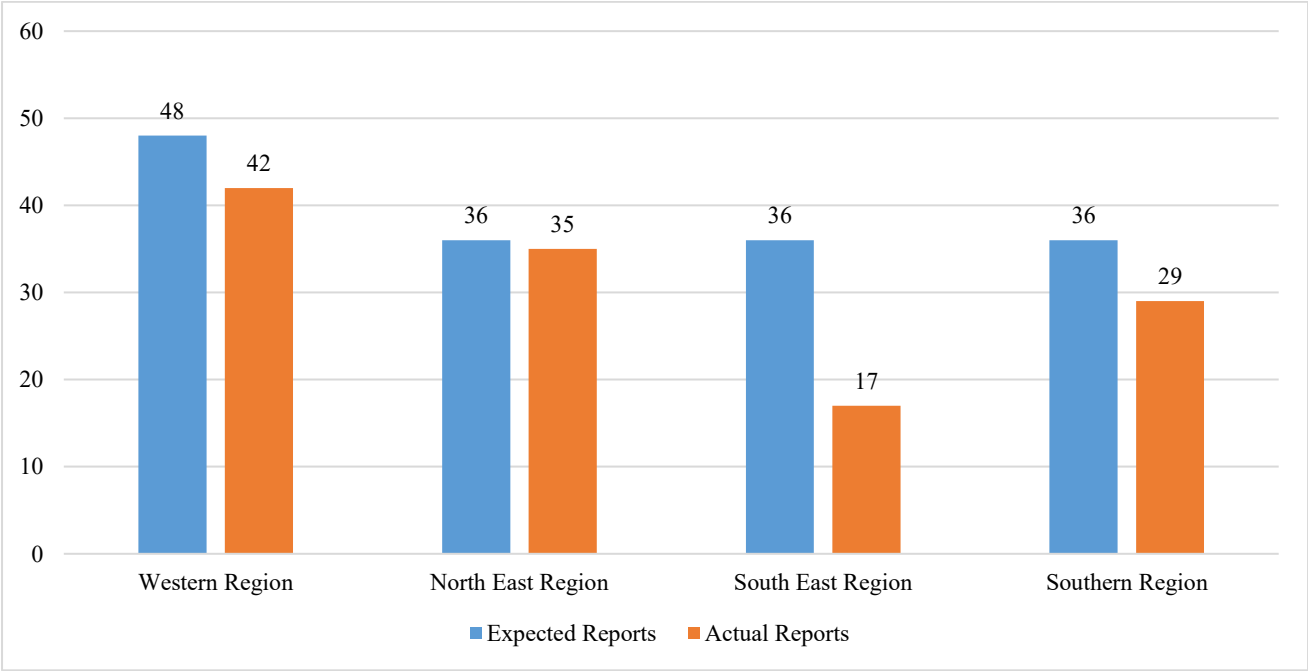


Figure 1. Actual Submitted versus Expected Reports by Regions (January - December 2025)

Completeness and Timeliness

The North East Region recorded 100% completeness across three quarters, from January to September, with all expected reports submitted. However, the completeness rate declined to 89% (8 of 9 reports submitted) during the October to December quarter (Figure 2). The Western Region recorded a completeness rate of 75% (9 of 12 reports) for the January to March and October to December quarters, and 100% (12 of 12 reports) for the April to September period. The Southern Region achieved a 100% completeness rate during the January to March quarter; however, the rate declined and remained constant at 67% (6 of 9 reports) for the period April to September and then slightly increased to 78% (7 of 9 reports) for the period October to December. The South East Region recorded the lowest completion rates for 2025, with a completeness rate of 44% (4 of 9 reports) for the January to March, July to September, and 33% (3 of 9 reports) for the October to December quarter. Its highest completeness rate was observed during the April to June quarter at 67% (6 of 9 reports submitted).

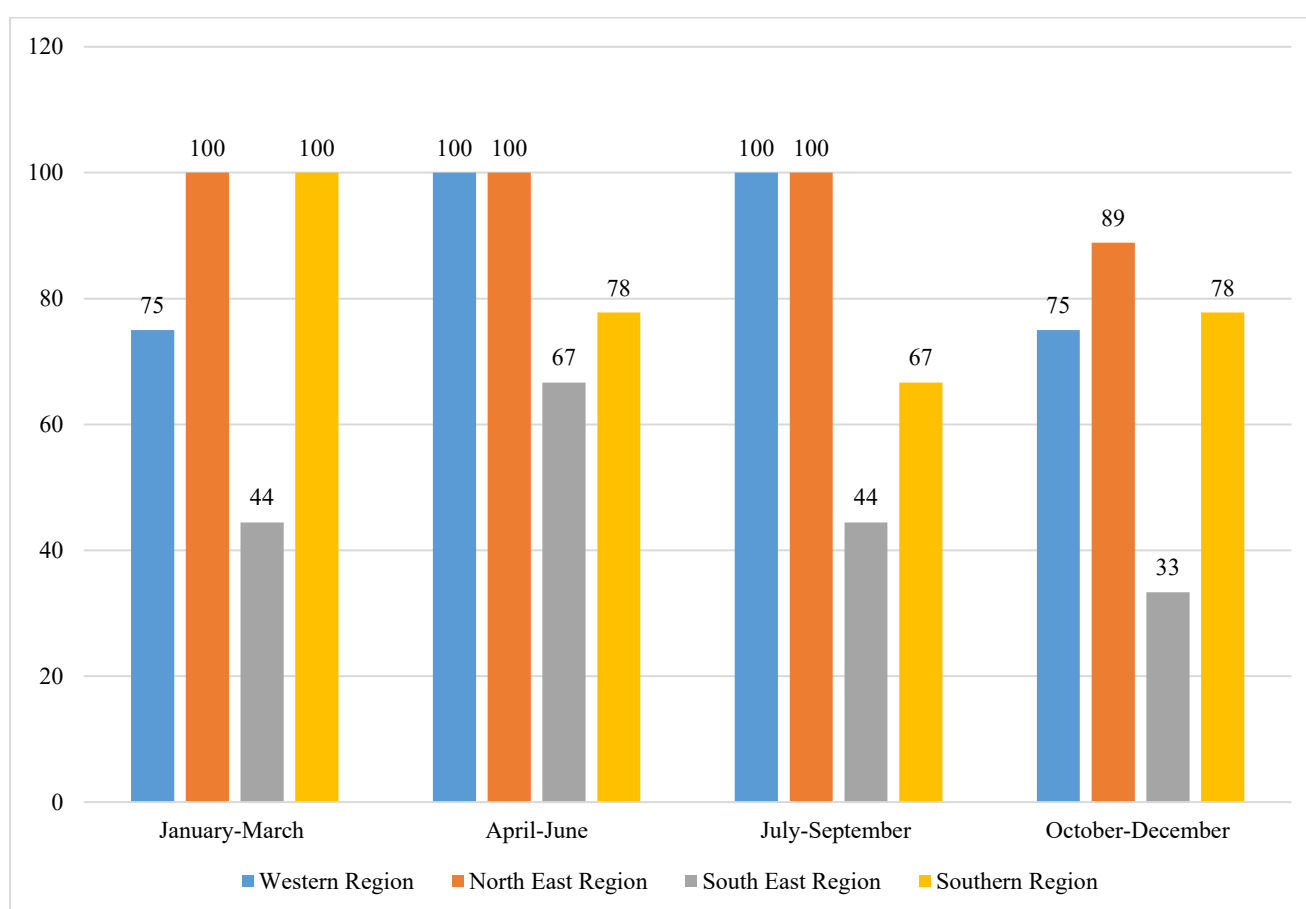


Figure 2. Completeness of Submitted Reports (January - December 2025)

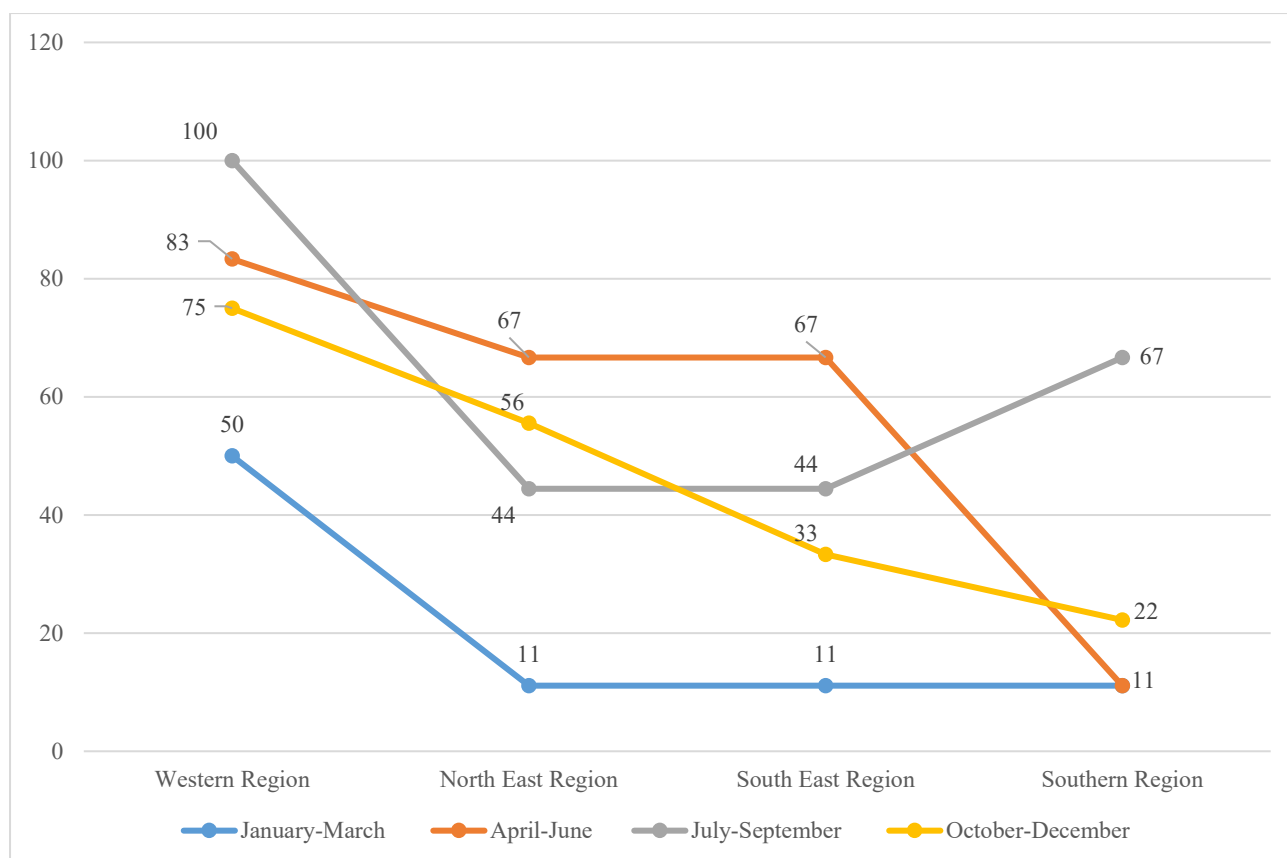


Figure 3. Timeliness of Submitted Reports (January - December 2025)

The timeliness for 2025 varied across regions and quarters (Figure 3). The Western Region demonstrated the strongest performance overall, with timeliness improving from 50% in the January to March quarter to 83% in April to June, peaking at 100% during the July to September quarter, before declining to 75% in October to December. The North East Region showed a gradual improvement in timeliness over the year, increasing from 11% in January to March to 56% by the October to December quarter, although performance remained below optimal levels throughout the year.

The Southern Region recorded low timeliness in the first half of the year, with 11% for both January to March and April to June. Performance improved substantially in July to September to 67%, followed by a decline to 22% in the October to December quarter.

The South East Region experienced fluctuations in timeliness, improving from 11% in January to March to 67% in April to June, before declining to 44% in July to September and further to 33% in October to December.

Database Analytics

Antenatal Care Coverage

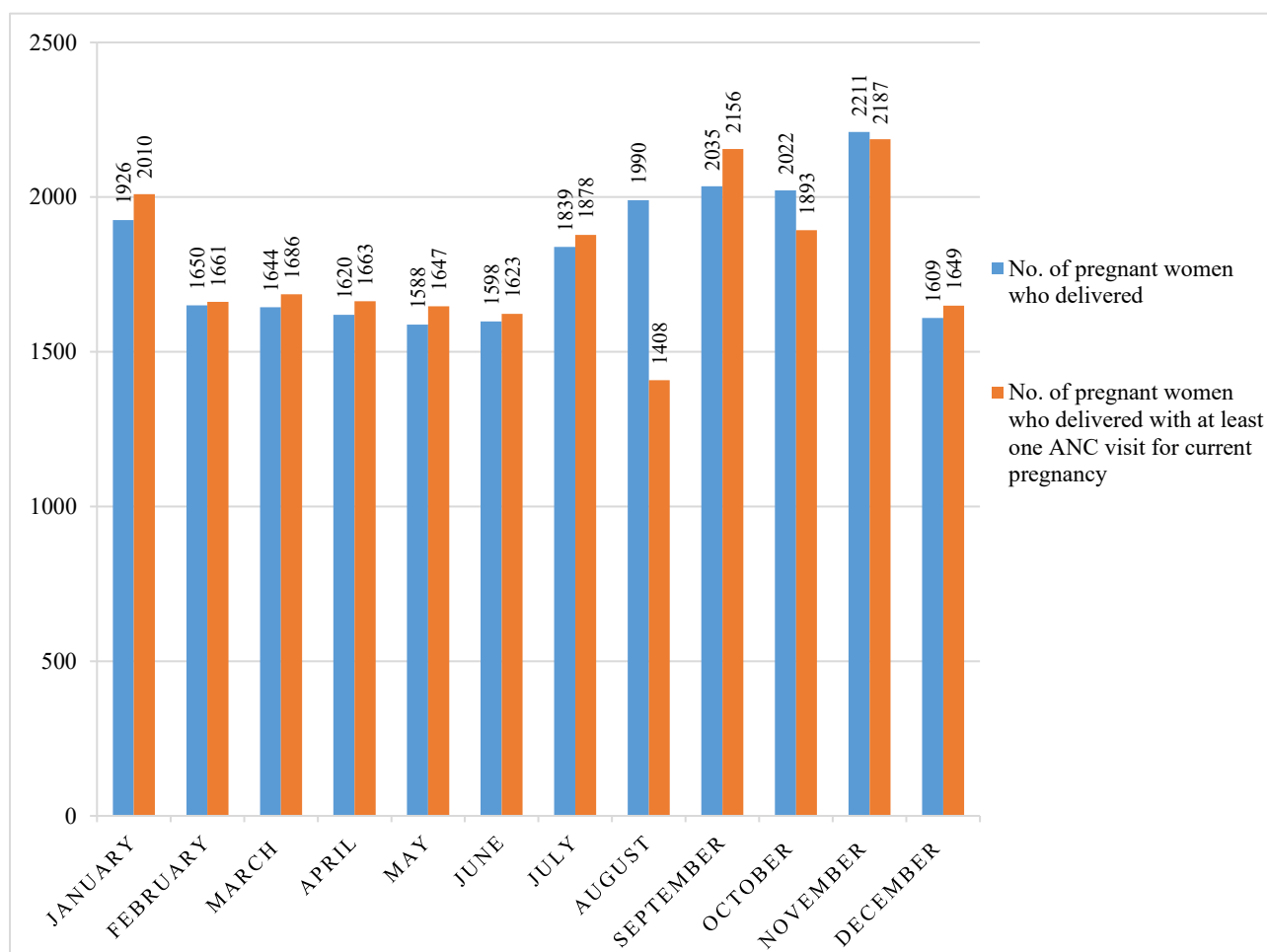


Figure 4. Antenatal testing coverage January-December 2025

Antenatal care coverage (ANC) among women who delivered during the reporting period remained near universal. Of the 21,732 recorded deliveries, 21,461 women had at least one documented ANC visit for the current pregnancy, corresponding to an overall coverage of 99%. ANC attendance was consistently high across most months, with modest variation observed. The highest volume of women accessing ANC services was noted in November (2,187 women), aligning with the peak in deliveries (2,211). In contrast, August recorded the lowest ANC coverage (71%), with 1,408 women receiving ANC among 1,990 deliveries. Notably there are months (August, October, November) where the ANC visit exceeds the number of women who delivered. This can be attributed to double counting or the inclusion of pregnant mothers who have not delivered. Overall, ANC attendance closely mirrored delivery numbers, reflecting a strong national PMTCT programme for pregnant mothers.

HIV Testing Coverage

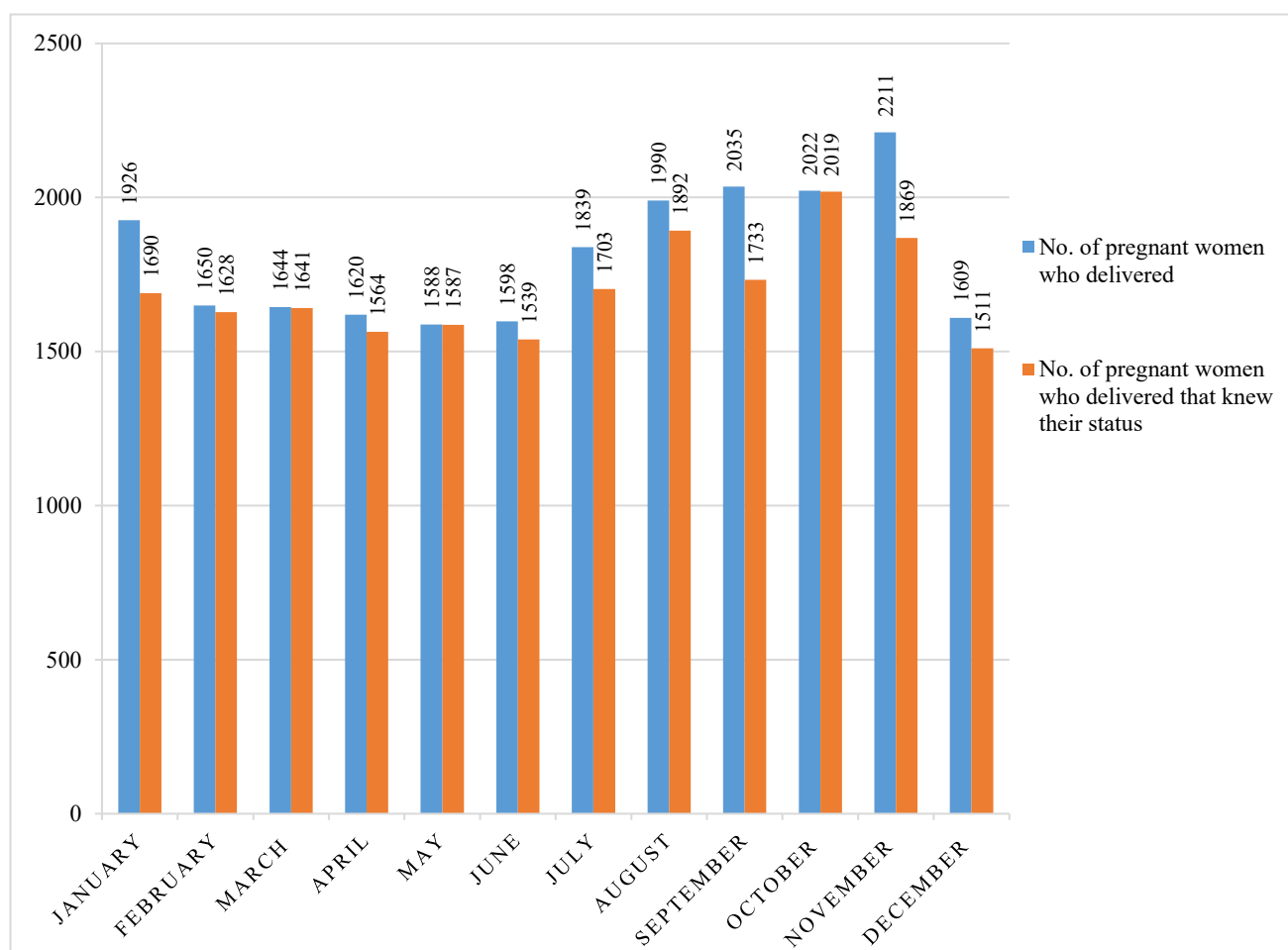


Figure 5. HIV testing coverage January-December 2025

A total of 21,732 pregnant women delivered across the reporting period. Among these, 20,376 women knew their HIV status at delivery, resulting in an overall testing coverage of 94% for the year. Monthly coverage was generally high, with small fluctuations. The highest coverage (99%) was observed in May (1,587 of 1,588), March (1,641 of 1,644) respectively. The lowest coverage (85%) was observed in November, where 1,869 women knew their status out of 2,211 who delivered, as well as September (1,733 of 2,035) reflecting a slight decrease compared to other months (due to low reporting rate). In most months, the number of women who knew their HIV status closely matched the number of deliveries. However, modest gaps highlight poor reporting rates or the need to strengthen data capture and ensure data completeness.

HIV Treatment Coverage

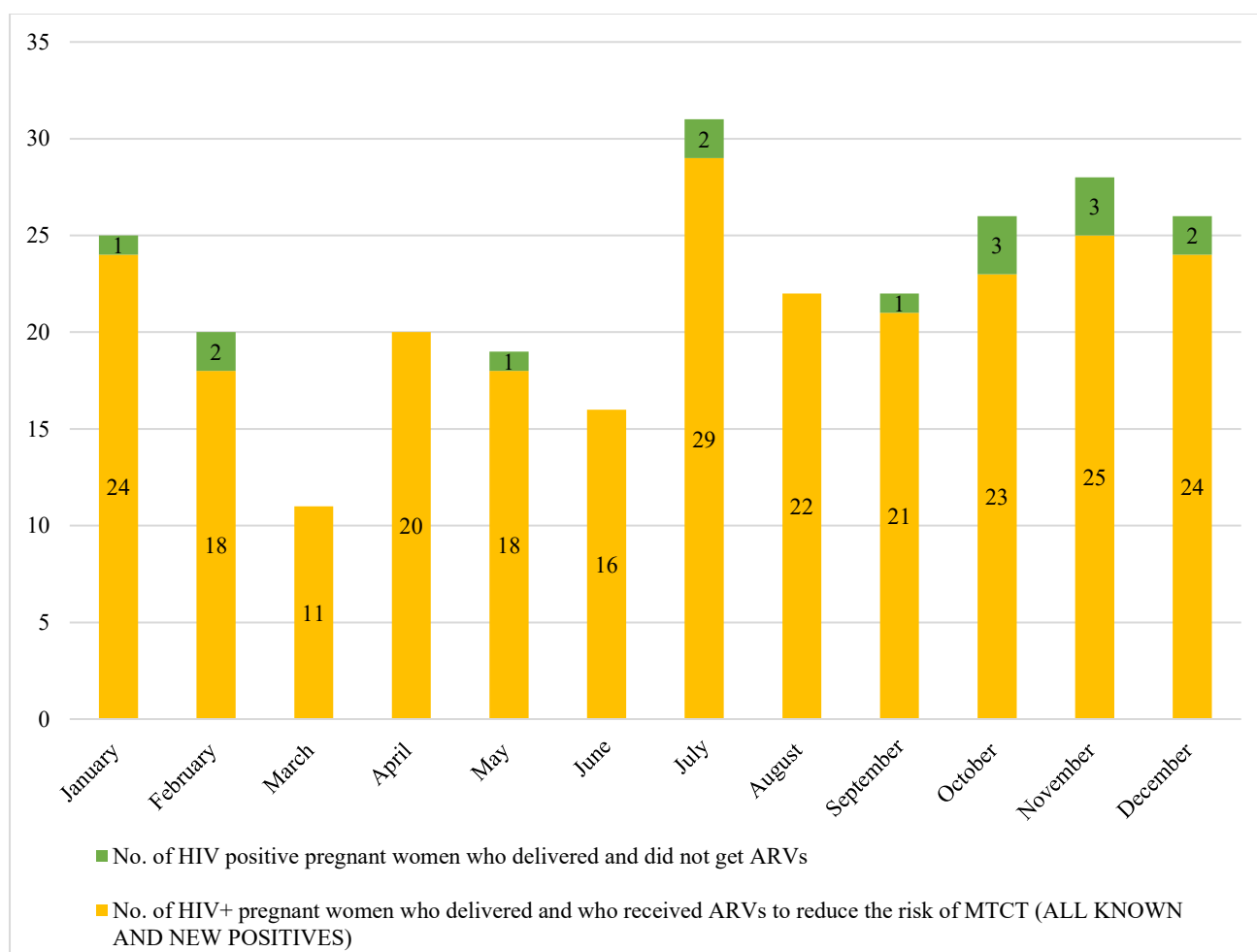


Figure 6. HIV treatment coverage January-December 2025

Antiretroviral coverage among pregnant women living with HIV who delivered during the year was largely comprehensive. Of the 266 women identified as HIV positive at delivery, 251 received ARVs (94% coverage) for the prevention of mother-to-child transmission, including both previously known and newly diagnosed cases. Provision of ARVs was consistently high across most months, with peak coverage recorded in March, April, June and August. However, 15 women delivered without receiving ARVs, with these instances occurring intermittently, particularly in February, October, November, and December. While the overall findings reflect effective PMTCT service delivery, the presence of untreated cases indicates ongoing opportunities to strengthen early case identification, linkage to treatment, and continuity of care during pregnancy.

Syphilis Testing Coverage

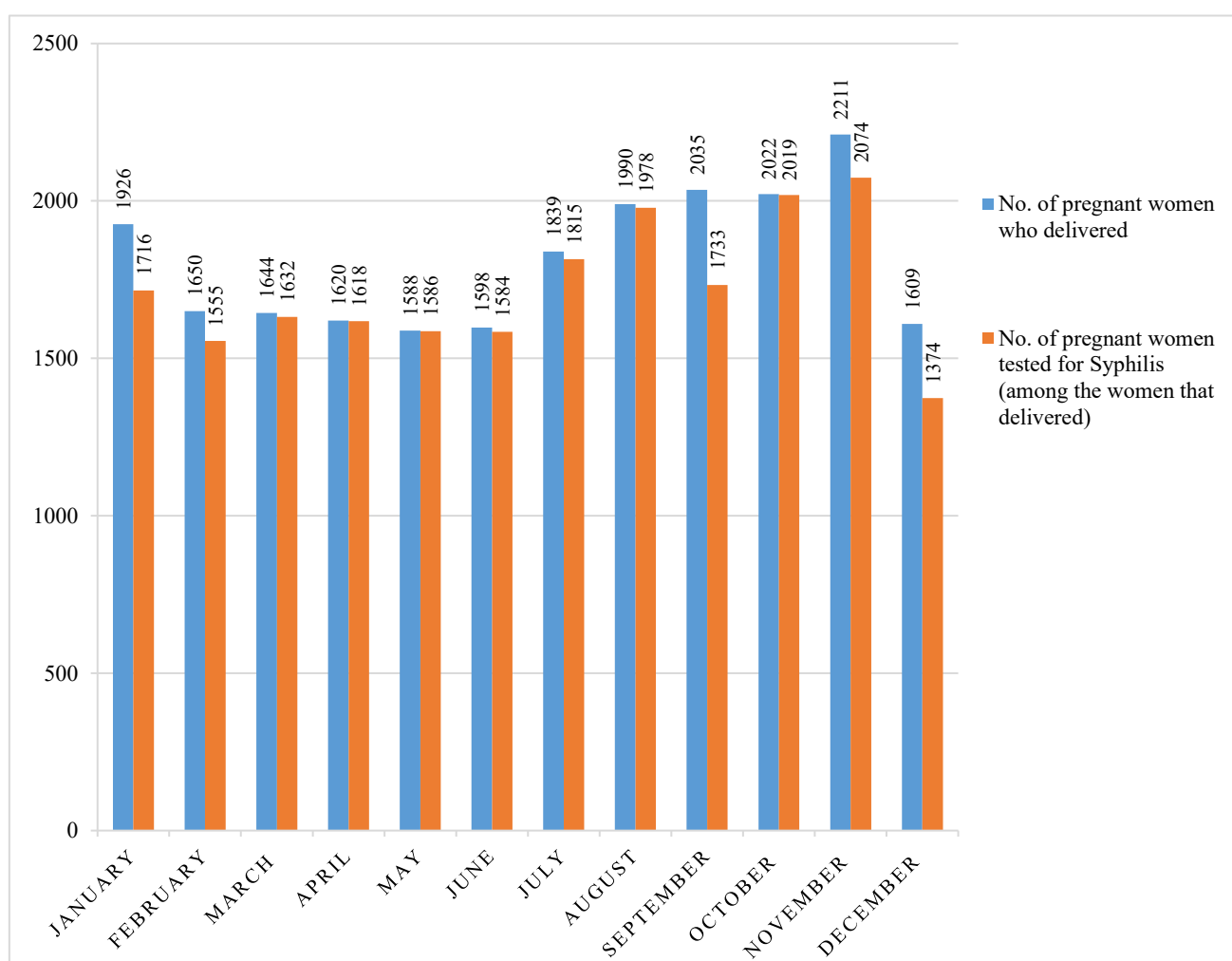


Figure 7. Syphilis testing coverage January-December 2025

A review of the database revealed that a total of 21,732 pregnant women delivered in 2025. Of this total, 20,684 women received syphilis testing, yielding an overall annual coverage of 95%. Syphilis screening remained consistently high throughout the year, with only slight month-to-month variations. Testing coverage was highest in April, of the 1,620 women who delivered 1,618 were screened corresponding to a 99% coverage. Coverage declined slightly in December, where 1,374 women were tested among 1,609 deliveries, suggesting a relative reduction in comparison to other months, which is associated with low reporting rates. For the majority of the year, syphilis testing closely aligned with delivery volumes, as observed in March and May. However, major discrepancies were identified in September and December warranting prompt reporting of complete data.

Syphilis Treatment Coverage

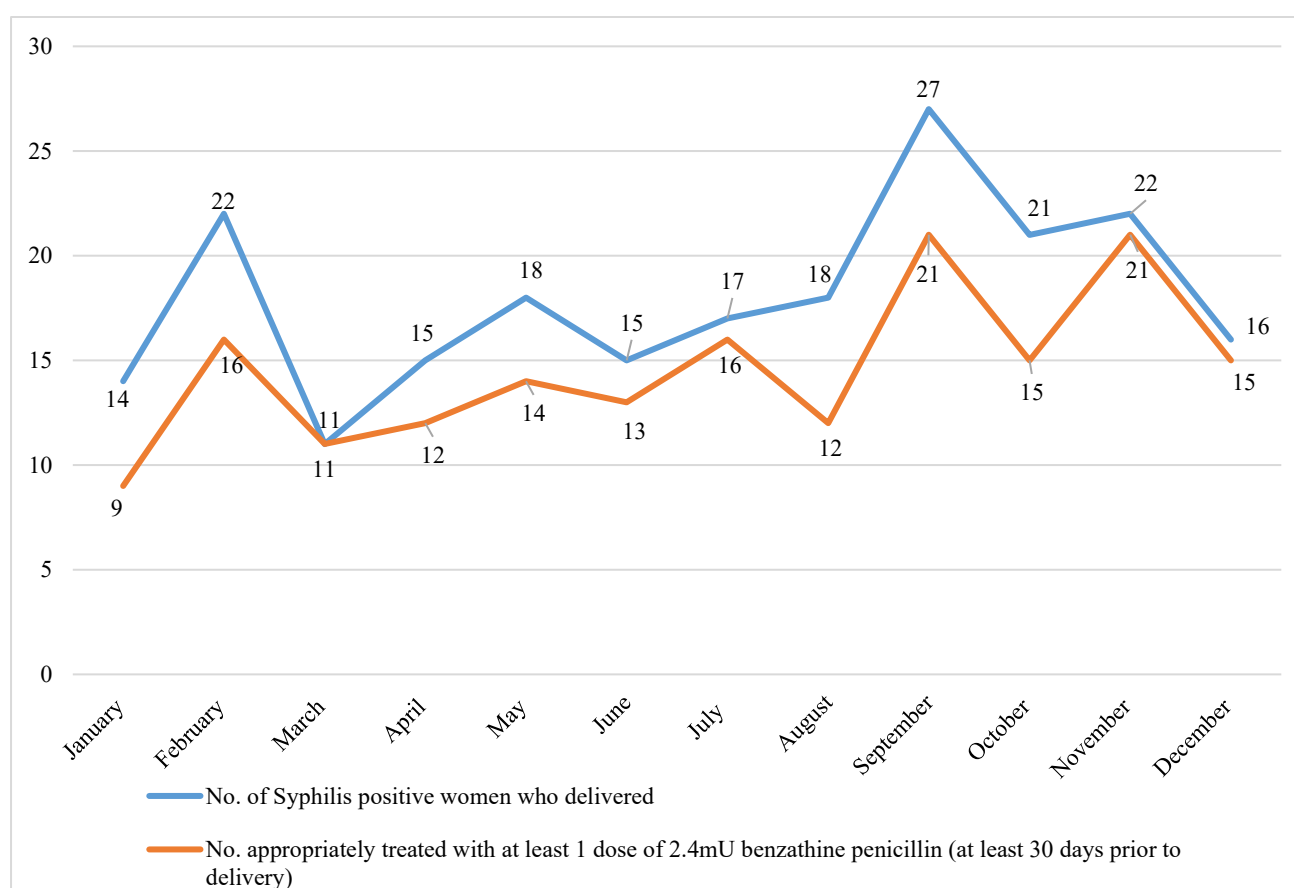


Figure 8. Syphilis treatment coverage January-December 2025

A total of 216 pregnant women who delivered were diagnosed with syphilis. Of these, 175 women received appropriate treatment, defined as at least one dose of 2.4 million units of benzathine penicillin administered a minimum of 30 days prior to delivery, resulting in an overall treatment coverage of 81%. Treatment uptake varied modestly across the months. The highest numbers of adequately treated women was recorded in March (11 of 11) and November (21 of 22), reflecting strong adherence to treatment national and international protocols. Lower treatment coverage was observed in January (9 of 14) and August (12 of 18), indicating gaps between diagnosis and the initiation of treatment services. Notably, 41 mothers were not adequately treated according to the database. Based on the PAHO/WHO definition of congenital syphilis, this corresponds to 41 cases of congenital syphilis in 2025, rather than the three cases currently indicated in the database. This highlights the need for adequate and timely treatment initiation is critical to the prevention of vertical transmission of syphilis.

National Impact Indicators

The table below presents national impact indicators for the elimination of mother-to-child transmission (EMTCT) of HIV and syphilis for January-December 2025. The HIV mother-to-child transmission (MTCT) rate was 1.1%, based on 3 infected infants out of a total of 270 HIV-exposed infant births, which is below the elimination target of less than 2%. Similarly, the rate of new HIV infections per 1,000 live births was 0.01%, with 3 new infections among 21,732 live births, substantially lower than the target of less than 0.3%.

The rate of congenital syphilis was 0.02%, calculated from 5 cases among 21,567 live births, also meeting the elimination benchmark of less than 0.5%. Overall, the findings indicate that the country can maintain its elimination status based on the data that is currently on the database.

Table 1. National Impact Indicators

IMPACT INDICATORS	Target	January-December 2025		
		Numerator	Denominator	Result (%)
HIV MTCT rate	<2%	3	270	1.1%
Rate of new infec. Per 1000 live births	<0.3%	3	21,732	0.01%
Rate of Congenital Syphilis per live birth	<0.5%	5	21,567	0.02%

Source: EMTCT database

Note: Due to low reporting rates, the data presented are incomplete and should be interpreted as a proxy dataset.

Discrepancies

During the reporting period, multiple discrepancies were identified across reporting sites, as presented in the tables and figures below. These included instances of inaccurate and incomplete reporting across several sections of the database. A common and persistent issue was the failure by most sites to record the number of first antenatal care visits (Indicator 4) and trimester specific testing for syphilis (indicator 7a).

General Hospital and Obstetrics Data

Indicators 1 to 3 capture data from general hospitals and obstetric services. A review of the reporting period identified several discrepancies across parishes with indicator #1a which looks at No. of pregnant women who delivered with at least one ANC visit for current pregnancy. Within NERHA and SRHA, no discrepancies were recorded for Portland, St. Ann, St. Mary, and St. Elizabeth during the review period. However, Manchester reported Indicator 1a incorrectly in August, while Clarendon did not submit any data for Indicator 1a in October and December, reflecting gaps in reporting completeness. However, more substantial reporting issues were observed. St. Catherine failed to submit reports for the period September through December, resulting in missing data across Indicators 1 to 44A (Table 2). Additionally, Kingston and St. Andrew (KSA) recorded no data for Indicator 1a in August, and St. Thomas similarly reported no data for Indicator 1a in November. Both Hanover and Westmoreland did not submit any reports in January and October, and subsequently Hanover incorrectly reported indicator 1a in November.

Table 2. General Hospital and Obstetrics data discrepancies

Region	Parish	Month (s)	Indicator #	Comment
NERHA	Portland	-	-	-
	St. Ann	-	-	-
	St. Mary	-	-	-
SRHA	St. Elizabeth	-	-	-
	Manchester	August	1a	Data incorrectly reported.
	Clarendon	October, December	1a, 1-44A	No data reported for 1a in October. No report was submitted in December.
SERHA	St. Catherine	September-December	1-44A	No report was submitted.
	KSA	August	1a	No data reported.
	St. Thomas	November	1a	No data reported.
WRHA	Trelawny	-	-	-
	St. James	-	-	-
	Hanover	January, October, November	1-44A, 1a	No report was submitted in January and October. Indicator 1a was incorrectly reported in November.
	Westmoreland	January, October	1-44A	No data reported.

Source: EMTCT Database

PMTCT Obstetric Data

PMTCT obstetric data are captured under Indicators 4 to 6 within the database. A review of the reporting period revealed several data quality discrepancies, most notably for Indicator 4, which records the number of pregnant women who had a first antenatal care (ANC) visit for the current pregnancy. This indicator was not reported using the required gestational age disaggregation (0-15 weeks, 16-28 weeks, and ≥ 29 weeks). Within NERHA, no discrepancies were observed for Portland and St. Ann during the review period; however, St. Mary did not report data for Indicator 5 in February and indicator 4 in November and December.

In SRHA, St. Elizabeth failed to submit data for Indicator 4 in March, while Manchester recorded multiple issues, including missing data for Indicator 4 and incorrectly reported data for Indicator 5 in February, with no data submitted for Indicator 4 in November and December. Clarendon did not submit any reports in December, resulting in missing data across Indicators 1 to 44A (Table 3).

More extensive discrepancies were observed in SERHA. St. Catherine failed to report data for Indicator 4 from February to August and submitted no reports from September to December. Kingston and St. Andrew did not report data for Indicators 4 and 6 for most of the year (January to March and May to December), while St. Thomas failed to submit data for Indicator 4 from February through November and indicator 5 in April.

Additionally, in WRHA Trelawny did not submit data for Indicator 4 from February to December. St. James had no data entered for Indicator 4 throughout the year and reported incorrect data for Indicator 5 in November. Both Hanover and Westmoreland did not submit any reports in January, and subsequently failed to report data for Indicator 4 from February to December, resulting in missing data across Indicators 1 to 44A for January.

Table 3. PMTCT Obstetric data discrepancies

Region	Parish	Month (s)	Indicator #	Comment
NERHA	Portland	-	-	-
	St. Ann	-	-	-
	St. Mary	February, November, December	4, 5	No data reported
SRHA	St. Elizabeth	March	4	No data
	Manchester	February, November, December	4,5	No data were reported for Indicator 4 in February, November, and December. Additionally, data for Indicator 5 were incorrectly reported in February
SERHA	Clarendon	December	1-44A	No report
	St. Catherine	February-August, September-December	4	No data were reported from February-August, and no reports were submitted from September-December
	KSA	January-March, May-December	4,6	No data reported
WRHA	St. Thomas	February-November	4,5	No data reported
	Trelawny	February-December	4	No data reported
	St. James	January-December	4, 5	No data were reported for Indicator 4, and data for Indicator 5 were incorrectly reported in November
	Hanover	January-December	1-44A, 4	No report was submitted in January, and data for Indicator 4 were missing from February through December.
	Westmoreland	January-December	1-44A, 4	No report was submitted in January, and data for Indicator 4 were missing from February through December

Source: EMTCT database

Syphilis Data

Indicators 7 to 11 capture syphilis-related data within the database. For the reporting period, data quality issues were minimal in NERHA; however, St. Ann reported Indicator 7 incorrectly in December (Figure 14), while no discrepancies were observed for Portland and St. Mary during the review period. St. Elizabeth reported incorrect data for Indicator 7a in February, while Manchester failed to submit any data for Indicator 7a in April. Clarendon had missing data for Indicators 8 and 9 in February and did not submit a report in December, resulting in missing data across Indicators 1 to 44A for that month. More extensive reporting gaps were identified in SERHA. St. Catherine did not report any data for Indicator 7a from April to August and failed to submit reports from September to December, resulting in missing data across Indicators 1 to 44A (Table 4). St. Thomas demonstrated inconsistent reporting, with Indicator 7 missing in February, March, and August, and Indicator 7a missing in April, July, and August. No discrepancies were identified for Kingston and St. Andrew during the review period.

Trelawny failed to report data for Indicator 7a in February. Hanover did not submit reports in January and October and did not report data for Indicator 7a in February, while Westmoreland did not submit reports in January and October and had missing data for Indicator 7a in February and March. No discrepancies were observed for St. James during the reporting period.

Table 4. Syphilis data discrepancies

Region	Parish	Month(s)	Indicator #	Comment
NERHA	Portland	-	-	-
	St. Ann	December	7	Data incorrectly reported
	St. Mary	-	-	-
SRHA	St. Elizabeth	February	7a	Data incorrectly reported
	Manchester	April	7a	No data reported
	Clarendon	February, December	8-9, 1-44A	No data were reported for Indicators 8 and 9 in February, and no report was submitted in December
SERHA	St. Catherine	April-August, September-December	7a, 1-44A	No data were reported for indicator 7a April- August. No report was submitted September-December
	KSA	-	-	-
	St. Thomas	February-April, July, August	7, 7a	No data were reported for Indicator 7 in February-March and August. No data were reported for Indicator 7a in April, July, and August
WRHA	Trelawny	February	7a	No data reported
	St. James	-	-	-
	Hanover	January, February, October	7a, 1-44A	No data were reported for Indicator 7a in February, and no reports were submitted in January and October
	Westmoreland	January-March, October	7a, 1-44A	No data were reported for Indicator 7a in February and March, and no reports were submitted in January and October

Source: EMTCT database

HIV Data

HIV-related data are captured through Indicators 12 to 15B within the database. A review of this section for the reporting period revealed several discrepancies in the reported data. In NERHA, St. Ann recorded

Indicator 12 incorrectly in September, while no issues were identified for Portland or St. Mary during the review period. For SRHA, reporting was generally consistent for St. Elizabeth and Manchester. However, Clarendon did not submit a report in December, resulting in missing data across Indicators 1 to 44A for that month.

More pronounced reporting gaps were observed in SERHA. St. Catherine failed to report data for Indicators 15 to 15B in March and did not submit any reports from September through December, leading to missing data across Indicators 1 to 44A for those months. No discrepancies were observed for Kingston and St. Andrew or St. Thomas during the reporting period. In WRHA, Hanover did not submit reports in January and October, while Westmoreland failed to submit a report in January, resulting in missing data across Indicators 1 to 44A for those periods. No issues were identified for Trelawny or St. James (Table 5).

Table 4. HIV data discrepancies

<i>Region</i>	<i>Parish</i>	<i>Month(s)</i>	<i>Indicator #</i>	<i>Comment</i>
NERHA	Portland	-	-	-
	St. Ann	September	12	Data incorrectly reported
	St. Mary	-	-	-
SRHA	St. Elizabeth	-	-	-
	Manchester	-	-	-
	Clarendon	December	1-44A	No report was submitted
SERHA	St. Catherine	March, September-December	15-15b, 1-44A	No data were reported for Indicators 15-15b in March, and no reports were submitted from September-December
	KSA	-	-	-
	St. Thomas	-	-	-
WRHA	Trelawny	-	-	-
	St. James	-	-	-
	Hanover	January, October	1-44A	No report was submitted
	Westmoreland	January	1-44A	No report was submitted

Source: EMTCT database

Syphilis Confirmed Infants (SCIs)

This section of the database captures information on syphilis-confirmed infants, represented by Indicators 16 through 20. A review of the reporting period identified data quality concerns primarily related to missing or incomplete entries. No data quality issues were observed in NERHA (Portland, St. Ann, St. Mary) and SERHA for St. Elizabeth, or Manchester. However, Clarendon did not submit a report in December. In SERHA, St. Catherine failed to submit reports from September through December, leading to the absence of data across Indicators 1 to 44A for the period (Table 5). No reporting issues were identified for Kingston and St. Andrew or St. Thomas.

Hanover and Westmoreland did not submit reports in January and October, resulting in missing data across Indicators 1 to 44A for those months for the Western Region. No discrepancies were noted for Trelawny or St. James.

Table 5. Syphilis Confirmed Infants data discrepancies

Region	Parish	Month(s)	Indicator #	Comment
NERHA	Portland	-	-	-
	St. Ann	-	-	-
	St. Mary	-	-	-
SRHA	St. Elizabeth	-	-	-
	Manchester	-	-	-
	Clarendon	December	1-44A	No report was submitted
SERHA	St. Catherine	September-December	1-44A	No report was submitted.
	Kingston & St. Andrew	-	-	-
	St. Thomas	-	-	-
WRHA	Trelawny	-	-	-
	St. James	-	-	-
	Hanover	January, October	1-44A	No report was submitted.
	Westmoreland	January, October	1-44A	No report was submitted.

Source: EMTCT database

HIV Exposed Infants (HEIs)

The HEIs section of the database is captured by indicators 21 to 36B. A review of this section identified several inconsistencies across parishes. No data quality issues were identified for Portland, St. Ann, St. Mary, St. Elizabeth, Manchester, St. Thomas, Trelawny, or St. James. In SRHA, Clarendon did not submit a report in December, resulting in missing data across Indicators 1 to 44A for that month. Several reporting gaps were observed in SERHA. St. Catherine did not report any data for Indicators 21 to 26 in March and failed to submit reports from September through December, leading to missing data across Indicators 1 to 44A for those months. In WRHA, Hanover did not submit a report in January, while Westmoreland failed to submit reports in January and October (Table 6).

Table 6. HIV Exposed Infants data discrepancies

Region	Parish	Month(s)	Indicator #	Comment
NERHA	Portland	-	-	-
	St. Ann	-	-	-
	St. Mary	-	-	-
SRHA	St. Elizabeth	-	-	-
	Manchester	-	-	-
	Clarendon	December	1-44A	No report was submitted
SERHA	St. Catherine	March, September-December	21-26, 1-44A	No data were reported for Indicators 21-26 in March, and no reports were submitted from September through December
	Kingston and St. Andrew	-	-	-
	St. Thomas	-	-	-
WRHA	Trelawny	-	-	-
	St. James	-	-	-
	Hanover	January	1-44A	
	Westmoreland	January, October	1-44A	No report was submitted

Source: EMTCT database

HIV Positive Children & Adolescents

This section of the database, represented by Indicators 37 to 44A, captures data on HIV-exposed and HIV-positive children. A review of the reporting period identified multiple data quality discrepancies across parishes. No discrepancies were observed for Portland, St. Mary, St. Elizabeth, Manchester, St. Thomas, Trelawny, or St. James during the review period. However, In SRHA, Clarendon did not submit a report in December, resulting in missing data across Indicators 1 to 44A for that month. St. Catherine in SERHA did not report any data for Indicator 37 in March and failed to submit reports from September through December, leading to missing data across Indicators 1 to 44A. Kingston and St. Andrew did not submit any reports from February to December, resulting in missing data across Indicators 37 to 44A for the entire period (Table 7). Hanover did not submit a report in January, while Westmoreland failed to submit reports in January and October.

Table 7. HIV Positive Children and Adolescents data discrepancies

<i>Region</i>	<i>Parish</i>	<i>Month(s)</i>	<i>Indicator #</i>	<i>Comment</i>
<i>NERHA</i>	Portland	-	-	-
	St. Ann			
	St. Mary	-	-	-
<i>SRHA</i>	St. Elizabeth	-	-	-
	Manchester	-	-	-
	Clarendon	December	1-44A	No report was submitted
<i>SERHA</i>	St. Catherine	March, September-December	37, 1-44a	No data reported for indicator 37 in March. No report was submitted September-December
	KSA	February-December	37-44A	No report was submitted
	St. Thomas	-	-	-
<i>WRHA</i>	Trelawny	-	-	-
	St. James	-	-	-
	Hanover	January	1-44A	No report was submitted
	Westmoreland	January, October	1-44A	No report was submitted

Source: EMTCT database

Recommendations to Address Discrepancies Seen in the PMTCT Database

Discrepancies & Challenges Identified	Interventions	Timeline	Responsible Persons
Missing or incorrectly reported data for Indicators 4-6 across multiple parishes and months	Implement routine data checks before submission.	Ongoing	PMTCT Coordinators & Medical Officers of Health
Indicators 7-9 and 7a not reported or incorrectly reported in multiple parishes	Enhance data capture for trimester-specific syphilis testing by coordinating with laboratories to indicate the corresponding trimester on test forms and results.	Ongoing	PMTCT Nurses
Indicators 12-15B: discrepancies between HIV-positive deliveries and ARV coverage; missing reports from multiple parishes	• Cross-check HIV-positive mother registers against ARV administration records • Implement mandatory internal monthly reporting deadlines	Ongoing	PMTCT Coordinators & Medical Officers of Health
Missing reports or incomplete entries from multiple parishes	Issue system reminders for late submissions	Ongoing	Mr. Sheldon Whorms
Multiple parishes failing to submit reports or submit incomplete reports	Introduce performance recognition and awards tied to timely and complete data submission	Ongoing	National Team
ANC visits exceeding the number of who delivered	Implement routine data verification activities prior to submission of reports	Ongoing	PMTCT Coordinators & Medical Officers of Health
Gaps at quarterly meetings	Reinforce review and feedback chain mechanism to ensure gap in data are identified and addressed before	Ongoing	Strategic Information Component
Low Syphilis Treatment coverage	Case review to carefully assess cases and classify them appropriately	As soon as possible	Treatment Care & Support component, Family Health Unit, PMTCT Nurses & Medical Officers of Health